

ScheduleAVisit.com

Weekly hour caps · Trusted peer referrals

Booking for independent counselors & therapists

Go-to-Market Marketing Plan

Founder-ready GTM · Version 1

Focus: Colorado first · Telehealth-friendly US states

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Founder

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scheduleavisit.com · Live demo: scheduleavisit.onrender.com

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Document meta

Product: scheduleavisit.onrender.com · Domain owned (DNS pending)

Founder: Jason Cheney · Date: August 22, 2026 · Status: Founder-ready GTM v1

1. Executive Summary

ScheduleAVisit.com helps solo and small-group clinicians protect their weekly caseload while keeping clients from falling through the cracks. Providers set a **weekly hour cap** (plus buffer). When they are full, the client is not turned away empty-handed — they see a **trusted peer referral** from the provider's own network.

This is **not** another EHR, insurance panel, or “get more clients” funnel. It is a **capacity + referral** product for clinicians who are already busy (or about to be) and who care about ethical handoffs.

Go-to-market thesis (2026)

Therapists discover practice tools through peers — Facebook private-practice groups, Reddit professional communities (r/therapists, r/psychotherapy, r/privatepractice), LinkedIn, CE webinars, consult groups, and coaches who already advise Headway/SonderMind clinicians. Cold ads are secondary. Trust, demo clarity, and referral etiquette win.

North-star outcome (first 90 days) — target, not a current metric

25–40 active Colorado / telehealth clinicians on the live app, with at least 8–12 using peer referral invites, and clear qualitative proof that “weekly cap + trusted handoff” resonates.

What we will not do: claim medical outcomes, collect or advertise PHI, spam Reddit, or compete as a full EHR. Honesty about current product limits (no HIPAA BAA yet, DNS pending, no transactional email) is part of the brand.

2. Positioning

Category

Capacity-aware booking with peer referrals — a lightweight scheduling layer for independent clinicians, not a replacement for TherapyNotes, SimplePractice, Headway, or Psychology Today.

Positioning statement

“For solo and small-group LPC / LCSW / LMFT / psychologists who want sustainable caseloads, ScheduleAVisit is the booking link that respects your weekly hour cap and, when you’re full, routes the client to a peer you actually trust — so good referrals stay in your network instead of disappearing into a directory.”

Competitive frame

They sell	We sell
More leads / more clients	Sustainable hours + ethical overflow
All-in-one EHR + billing	Simple public booking + capacity math
Marketplace matching strangers	Your own trusted peer network
“Never turn clients away” pressure	Permission to be full, gracefully

Proof points (use only what is true)

- Live demo: <https://scheduleavisit.onrender.com>
- Server-side weekly capacity (target hours + buffer + recurring load)
- Peer invite / accept referral network
- Consult vs session booking flow
- Optional iCal busy import
- Founder is a practicing clinician (Jason Cheney) — not a VC-backed SaaS outsider

Explicit non-claims

- Not HIPAA-certified / no BAA yet
- Not a clinical decision tool
- Not insurance billing
- Custom domain owned but not yet pointed

3. Ideal Customer Profile (ICP)

Primary ICP (beachhead)

Solo private-practice clinician in Colorado (or PSYPACT / telehealth-friendly state)

- Licenses: LPC, LCSW, LMFT, psychologist
- Caseload: 15–28 clinical hours/week (or intentionally capping there)
- Pain: calendar chaos, guilt turning people away, informal “text a colleague” referrals that get lost
- Tools today: Psychology Today + Google Calendar / SimplePractice / TherapyNotes / Headway or Alma for insurance pieces
- Mindset: values boundaries, peer consult culture, not hustle-bro marketing

Secondary ICP

- 2–6 clinician group practices sharing overflow
- University counseling / master’s grads launching private practice who need a clean booking link + early referral circle
- Clinicians full on Headway/SonderMind who still want a private-pay or specialty booking page with trusted handoffs

Anti-ICP (do not chase yet)

- Large multi-site clinics needing enterprise EHR
- Practices requiring HIPAA BAA before any pilot
- Pure lead-gen / “fill my calendar at any cost” buyers
- Non-clinical coaches unless they clearly sit adjacent to licensed care

Jobs to be done

- **1.** Protect my weekly hour target without looking unavailable forever.
- **2.** When I’m full, send the client to someone I vouch for.
- **3.** Give clients one calm public link instead of email tennis.
- **4.** Keep overflow inside my peer circle, not a random directory.

4. Messaging Pillars

Pillar 1 — Sustainable hours

Headline: Set your weekly hour cap. Keep your life.

Proof: Target hours + paperwork/emergency buffer; capacity computed before a booking is allowed.

Tone: Calm, boundary-affirming, clinician-to-clinician.

Pillar 2 — Trusted handoffs

Headline: Full shouldn't mean "sorry, try Google."

Proof: Invite peers; when you're at capacity, clients see one recommended colleague first.

Tone: Ethical, relational, anti-abandonment.

Pillar 3 — Simple public link

Headline: One link. Consult or session. Done.

Proof: Public pages; returning clients book sessions; optional portal link after first booking.

Tone: Non-technical, founder-friendly.

Pillar 4 — Built with clinicians, not for "users"

Headline: Made for independent counselors — by someone who sits in the chair.

Proof: Founder story; Colorado beachhead; peer consult culture.

Tone: Humble, transparent about what's live vs. next.

Voice guidelines

- Prefer "clinician / counselor / therapist" over "provider user."
- Prefer "weekly cap / capacity" over "optimization / growth hacking."
- Prefer "trusted peer" over "network effects."
- Never imply ScheduleAVisit replaces clinical judgment or crisis care.
- Always offer a path to emergency resources when discussing overflow (high-level only).

Sample one-liners

- "Booking that respects your weekly hour cap."
- "When you're full, clients meet a peer you trust."
- "The anti-burnout booking link for private practice."
- "Keep good referrals inside your consult circle."

5. Channel Playbooks

Research basis (2026): therapists discover tools via peer communities more than cold ads. Facebook private-practice groups, Reddit clinician subs, LinkedIn, CE/listservs, and consult networks outperform broad Instagram/TikTok for *practice software* acquisition. Client-facing social still matters for *their* practices — not for selling ScheduleAVisit.

5.1 Reddit playbook

Where: r/therapists (~139k+), r/psychotherapy, r/privatepractice, r/socialwork (selectively), r/counseling. Avoid pitching in client-facing mental-health subs.

Rules of engagement

- Create a personal account (Jason, with transparent bio: clinician / founder). No corporate spam account.
- Spend 2–3 weeks answering questions with zero product links (caseload boundaries, referral ethics, scheduling friction).
- Follow the **80/20 or 90/10 value rule**: help first; mention the product only when directly relevant and invited.
- Read each sub's sidebar before posting. Many ban self-promo or require "Seeking Feedback" flair.
- Never cross-post the same link to multiple subs the same day.
- When sharing: lead with the problem and invite critique. Soft CTA to the Render demo.
- Offer free pilot seats to Reddit clinicians who give candid feedback.

Content that works

- "How do you handle overflow when your week is capped?"
- AMA-style: "I built a capacity-aware booking tool — roast my assumptions"
- Lessons from peer-referral failures (no PHI, no client stories)

What fails: drive-by links, "check out my SaaS," medical claims, arguing with mods.

5.2 Facebook Groups playbook

Where (examples to join & contribute):

- Private Practice Startup / Abundance Practice Building style groups
- Million Dollar Private Practice Community
- My Private Practice Collective
- Mental Health Business Development (therapist-only)
- Colorado / Front Range therapist networking groups
- Niche specialty groups (trauma, couples, teens) for soft relationship-building — not hard sells

Cadence: 4–5 genuine comments/week; 1 value post every 1–2 weeks max. Product mention only in comments when someone asks for scheduling tools, or via admin-approved resource threads.

Assets: 60-second screen demo (Loom), one-pager PDF, waitlist/email capture, Colorado pilot invite.

Etiquette: Ask admins before pinning or promoting. Lead with generosity. Never scrape member lists.

5.3 LinkedIn playbook

Who: Jason's personal profile as primary channel; company page secondary. Audience: clinicians, group practice owners, clinical supervisors, university faculty, Headway/SonderMind coaches, CE organizers.

Tactics

- 2 posts/week: boundaries, capacity math (non-PHI), founder build-in-public, Colorado mental-health workforce notes.
- Comment thoughtfully on posts from practice coaches and association leaders.
- Join / engage clinician LinkedIn groups focused on private practice marketing & management.
- DM only after public engagement; short problem statement + offer of a 15-min walkthrough — no pitch deck spam.
- Publish one long-form article in days 30–60: “Why ‘always accepting new clients’ burns therapists out.”

5.4 Instagram & TikTok playbook

Role: brand awareness and founder visibility — **not** primary SaaS acquisition.

Instagram: carousel explainers (“3 ways therapists handle being full”), Reels of calm UI walkthroughs, Stories polls. Aesthetic: cream / forest / terracotta; soft lighting; no stock “sad client” imagery.

TikTok: only if Jason is comfortable on camera. Topics: private-practice boundaries, “what I tell clients when my week is full,” tool teardowns. Avoid clinical advice and crisis content that attracts the wrong engagement.

Frequency: 3 IG posts/week or skip entirely until Reddit/FB/LinkedIn are humming. Do not dilute effort.

5.5 Email, listservs & CE webinars

Owned email

- Simple Beehiiv / Buttondown / Mailchimp list: “Capacity Lab” monthly note (boundaries, referral ethics, product updates).
- Lead magnet: “Weekly Hour Cap Worksheet” (PDF) — no PHI collection beyond email.
- Onboarding drip for pilots: day 0 setup, day 3 invite a peer, day 7 capacity check-in.

Listsers / associations

- Colorado Counseling Association (CCA) member channels & CCACE CE audiences
- NASW Colorado, AAMFT Colorado, Colorado Psychological Association touchpoints
- University counseling program alumni lists (DU, Regis, CU, Naropa, etc.) via faculty relationships — ask permission; no purchased spam lists

CE / webinars

- Offer a free 45-min CE-adjacent talk (ethics of referrals / caseload sustainability) and soft-mention the tool at the end. Partner with existing CE providers rather than claiming CE credit until accredited.

- Guest on private-practice podcasts (Therapy for Therapists, Private Practice skills shows).

5.6 Partnerships playbook

Priority partners

- **Practice coaches** who advise Headway / SonderMind / Alma clinicians — offer affiliate credit or free seats for their cohorts.
- **Peer consult group facilitators** (CAP peer consult, Clinical Collective–style Northern Colorado groups, Firelight-style networks) — complements “who do I send this to?” conversations.
- **Supervision / group practice consultants** — overflow infrastructure for supervisees launching private practice.
- **Complementary tools** (not competitors): intake form tools, website builders for therapists, local answering services — reciprocal resource pages.
- **Directories (Psychology Today, etc.):** do **not** compete; teach clinicians to keep PT for discovery and ScheduleAVisit for capacity-aware booking + handoff.

Partnership offer template

“Free pilot for your next cohort (10 seats). 20-min live demo. We never ask for client data. You get a unique invite link and we share anonymized learnings.”

5.7 Local / Colorado playbook

Why Colorado first: founder presence, strong private-practice density (Denver / Boulder / Fort Collins / Colorado Springs), active associations, telehealth culture, and demo seed data already oriented to Front Range peers.

Tactics

- Attend / sponsor lightly: CCA Annual Conference (Sept 2026 Denver), local networking breakfasts, university career panels.
- Coffee chats: 10 Colorado clinicians in 30 days (in-person or Zoom). Goal = feedback + first referral edges.
- Local Facebook / Meetup therapist groups; Chamber / wellness co-working boards.
- PSYPACT / multi-state telehealth clinicians living in CO as expansion bridges.
- Campus pipeline: offer a “Launching Private Practice” office-hours session for graduating cohorts.

6. 30-60-90 Day Plan

Days 1–30 — Foundation & listening

Product / ops

- Point scheduleavisit.com DNS to production (or clear redirect plan).
- Confirm demo accounts & public story match marketing claims.
- Create Loom 90-second demo + 1-page PDF.
- Set up analytics (Plausible or GA4) + simple signup source field.
- Start email list + Hour Cap Worksheet lead magnet.

Distribution

- Jason LinkedIn profile refresh (headline, featured demo link).
- Join 8–10 Facebook clinician groups; comment only for 2 weeks.
- Begin Reddit value-posting (no links week 1–2).
- Book 8–10 Colorado clinician conversations.
- Identify 5 practice coaches / consult facilitators for partner outreach.

Success criteria (targets): 10 qualitative interviews logged; demo link shared in ≥ 3 communities with positive (or constructive) feedback; email list ≥ 50 .

Days 31–60 — Soft launch & pilots

- Public “soft launch” post across LinkedIn + 2 FB groups (admin OK) + Reddit feedback thread.
- Onboard 15–25 pilot clinicians (Colorado-weighted).
- Require each pilot to invite ≥ 1 peer (tests referral loop).
- Run one free webinar: “Ethical overflow when your week is full.”
- Publish LinkedIn long-form + 4 educational carousels.
- First partner cohort (coach or consult group) live.

Success criteria (targets): 15+ activated accounts; ≥ 8 peer invites accepted; 3 written testimonials (non-PHI); webinar ≥ 25 attendees.

Days 61–90 — Repeatable motion

- Double down on top 2 channels by cost-per-activated-clinician.
- Formalize referral partner one-pager + tracking links.
- Second webinar or CCA/listserv feature.
- Content library: 6 evergreen posts + worksheet + demo.
- Decide: expand to one more telehealth-friendly state cluster **or** deepen Colorado density.
- Scope next product trust items (transactional email, domain polish, HIPAA roadmap messaging — carefully).

Success criteria (targets): 25–40 active clinicians; clear channel ROI; documented playbooks Jason can run weekly in ≤ 8 hours.

7. Sample Content Calendar (4 weeks)

Illustrative sample only — adjust cadence to Jason's available hours (~6–10 hrs/week marketing).

Week	LinkedIn	Facebook groups	Reddit	Email / other
1	"My weekly hour target (& why I defend it)"	Comment on 5 overflow / burnout threads	Answer 5 caseload questions (no links)	Setup Beehiiv; draft worksheet
2	Short demo Loom + "roast my UX"	Poll: "What do you say when you're full?"	Helpful comment thread; profile link only	Colorado coffee chat #1–4
3	Carousel: 4 ethical handoff scripts	Value post + soft pilot invite (admin OK)	Feedback post with demo (if karma ready)	Webinar landing page live
4	Article outline teaser	Share webinar	Recap learnings from pilots	Webinar + pilot onboarding drip

Always-on assets

- 90-sec demo video
- Hour Cap Worksheet PDF
- One-pager (problem / how it works / what's not included)
- FAQ: HIPAA status, data collected, crisis disclaimer

8. KPIs & Measurement

All numbers below are **example 90-day targets**, not current performance. Track honestly; revise after first interviews.

Funnel metrics

Stage	Metric	90-day target (example)
Awareness	Unique demo visits / mo	800–1,500
Interest	Email signups / worksheet downloads	150–300
Activation	Clinician accounts with setup complete	25–40
Habit	Accounts with ≥ 1 public booking or manual block	15–25
Network	Peer invites sent / accepted	20 sent / 10 accepted
Advocacy	Testimonials or case notes (non-PHI)	3–5

Channel health

- Cost per activated clinician (cash + estimated time)
- % of signups with known source (UTM / “how did you hear”)
- Reddit: karma trend + zero mod removals
- FB: comment-to-DM ratio (quality > reach)
- LinkedIn: profile views & connection accept rate from clinicians

Qualitative

- “Would you miss this if it went away?” (interview score)
- Objections log (HIPAA, email, domain, iCal, mobile) feeding product backlog — without modifying the app in this marketing workflow

9. Budget Starter (first 90 days)

Conservative founder-led budget. Time is the scarce resource. Figures are estimated ranges, not commitments.

Item	Est. monthly	Notes
Domain / hosting (mostly covered)	\$0–25	Render + domain renewal
Email tool	\$0–30	Free tier OK at start
Analytics	\$0–10	Plausible trial or GA4
Loom / design (Canva Pro)	\$0–15	Optional
Boosted LinkedIn or Meta tests	\$50–150	Only after organic proof; no PHI in creatives
CCA / local event (one-time)	\$100–400	Conference or meetup sponsor lite
CE webinar tools (Zoom)	\$0–20	
Misc (print one-pagers, coffee chats)	\$50	
Total cash	~\$100–400 / mo	Plus Jason's time (~6–10 hrs/week)

Do not spend yet: agencies, broad Google Ads for “therapy scheduling software,” TikTok ads, purchased email lists.

ROI mindset: one retained clinician who invites two peers can outperform \$500 in ads. Optimize for referral edges.

10. Risks & Compliance

Non-negotiables

- 1. No PHI in ads, posts, screenshots, or testimonials.** Use fictional demo names only (Elena / James / Maya style). Blur calendars. Never share real client details — even “successfully.”
- 2. No medical / outcome claims.** Do not say ScheduleAVisit improves clinical results, reduces suicide risk, or replaces crisis protocols.
- 3. Crisis posture.** When discussing overflow, remind clinicians that emergency and crisis pathways stay with their professional judgment and local resources — the product is scheduling + peer referral, not triage.
- 4. Honest product status.** No HIPAA BAA yet; say so. Do not market as a HIPAA product.
- 5. Reddit / FB etiquette.** Value-first; follow group rules; accept “no” from mods gracefully. One ban can close a channel for months.
- 6. FTC / advertising truthfulness.** Testimonials must be real and permissioned; disclose pilot/free access if relevant.
- 7. Data minimization.** Marketing forms collect email + optional license/state only. No clinical questionnaires for acquisition.
- 8. Platform ad policies.** Health-adjacent Meta/Google ads can be restricted — prefer organic clinician communities first.
- 9. Trademark / partnership language.** Do not imply endorsement by Headway, SonderMind, Psychology Today, or CCA without written OK.
- 10. Accessibility & inclusion.** Imagery and copy should welcome diverse clinicians; avoid stigma stereotypes.

Reputation risks

- Looking like “another tech bro scheduling app” → counter with founder clinician story + capacity ethics.
- Over-promising EHR features → keep scope tight in every asset.
- Spamming peer groups → irreversible trust loss; move slowly.

11. Next Actions for Jason

This week

- 1. Record a 90-second Loom demo of the live Render app (cap + full → peer referral path).
- 2. Refresh LinkedIn headline/about; pin the demo.
- 3. Join 8 clinician Facebook groups; set a daily 15-minute comment block.
- 4. Create the Hour Cap Worksheet (1 page) and email signup.
- 5. List 15 Colorado clinicians / supervisors to invite to coffee or Zoom.

This month

- 6. Point scheduleavisit.com (or publish a clear “live at ...” banner until DNS is ready).
- 7. Soft-launch feedback thread on Reddit after 2 weeks of helpful comments.
- 8. Book one practice coach or consult-group facilitator partnership call.
- 9. Schedule the “Ethical overflow” webinar date.
- 10. Log every objection in a simple spreadsheet (source, quote, severity).

Decision checkpoints

- **Day 30:** Are conversations energetic? If not, rewrite positioning with interview language.
- **Day 60:** Which single channel produced activated clinicians cheapest? Cut the rest temporarily.
- **Day 90:** Colorado density vs. next-state expansion.

Appendix A — Research Notes

Channels therapists actually use for practice-tool discovery (qualitative synthesis, 2026):

- **Peer communities dominate** tool discovery for private-practice software; Psychology Today remains #1 for *client* acquisition, not for *practice tools*.
- **Facebook Groups** (Private Practice Startup, Abundance Practice Building, Million Dollar Private Practice, therapist-only business development groups) are high-trust, high-etiquette spaces.
- **Reddit** r/therapists and related pro subs reward expertise; self-promo without karma is rejected; helpful threads gain long-tail Google visibility.
- **LinkedIn** works for B2B authority with supervisors, coaches, and group owners.
- **Instagram/TikTok** help personal brand; weak primary channel for clinical SaaS unless founder is already strong on camera.
- **CE webinars, listservs, peer consult groups, and university pipelines** convert because they mirror how clinicians already learn.
- **Headway / SonderMind adjacency:** partner with coaches and clinicians inside those ecosystems; never imply official partnership. Those platforms solve insurance access; ScheduleAVisit solves capacity + trusted overflow on the clinician's own link.
- **Colorado:** CCA conference, CAP peer consult, Clinical Collective—style Northern Colorado groups, and Front Range university programs are concrete local levers.

Appendix B — Brand

Token	Hex	Use
Cream	#F7F1E8	Page backgrounds, soft panels
Forest	#2F5D50	Primary text, headers, buttons
Terracotta	#C46D52	Accents, CTAs, highlights

Typography: clean sans for UI/PDF body; restrained serif optional for cover only. Plenty of whitespace. No stock “brain glow” clipart.

Prepared for Jason Cheney · ScheduleAVisit.com · August 22, 2026
This document is a marketing plan only. It does not modify the web application.